

5 June 2023

Lucy Letby - Cross-examination Day 7: Babies K, F & L
Juror sent home unwell

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MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

Housekeeping

Monday, 5 June 2023
(10.30 am)

(Delay in proceedings)

(10.35 am)

(In the absence of the jury)

MR JUSTICE GOSS: I'm sorry about the slight delay. On Wednesday, I'll say this now and I'll say it to the jury when they come in, we'll finish no later than 3 pm: a juror has a medical appointment. Thank you.

(In the presence of the jury)

MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

MR JUSTICE GOSS: Good morning. Sorry about the slight delay.

On Wednesday we will be finishing no later than 3 o'clock in the afternoon. All right? I'm aware of the other issue which affects one of you and we'll wait and see what happens in relation to that. Thank you very much.

Yes, Mr Johnson.

MR JOHNSON: Thank you, my Lord.

Just having had the opportunity over the weekend to think about what it is you've said on Friday, and indeed before that, is there anything that you would like to review?

LETBY: No.

JOHNSON: If we go to tile 104, please, in the [Baby K] sequence. This is something that we touched on briefly on Friday. This was your admission record for [Baby K]; is that right?

LETBY: Yes.

JOHNSON: The record shows that it was opened at 06.04 and ended at 06.10. We can see that second time there at the top of the page.

LETBY: Yes.

JOHNSON: I asked you about the care of the endotracheal tube, which of course was highly relevant to [Baby K], wasn't it?

LETBY: Yes.

JOHNSON: Did you insert that data, "Patient has an artificial airway"?

LETBY: Yes, so part of the -- to obtain that care plan you have to tick certain boxes of equipment that the baby has attached to them, yes.

JOHNSON: And as a result of you ticking that box, what did the machine then produce in terms of information?

LETBY: The parts underneath.

JOHNSON: Yes. And what are they, please?

LETBY: "Secure ETT to taped bonnet. Retie tapes with two nurses in case of accidental extubation. Record tube size and length. Monitor patient's tolerance of ET tube to aid compliance."

JOHNSON: And we know that within 5 minutes of you entering that data, [Baby K]'s tube displaced again, don't we?

LETBY: Yes.

JOHNSON: Yes. And that you were with [Baby K] between you closing that entry and her desaturating again?

LETBY: I can't say that, no.

JOHNSON: Why can't you say that?

LETBY: Because I've no memory of being at [Baby K]'s cot side.

JOHNSON: Right. Well, what would you normally do with the records once you'd booked her in?

LETBY: Take them back to the nursery or give them to the member of staff looking after the baby.

JOHNSON: You'd put them back at the cot side, wouldn't you?

LETBY: Yes.

JOHNSON: Yes.

LETBY: Or give them to a member of staff, yes.

JOHNSON: This was [Baby K]'s second desaturation. We'll come to the first at the end. Do you remember me explaining that to you on Friday?

LETBY: Yes.

JOHNSON: You will remember, I'm sure, that a morphine infusion had been running since 03.50 that morning.

LETBY: Yes.

JOHNSON: And so by the time [Baby K] desaturated on this second occasion, she was well sedated; would you agree?

LETBY: Yes.

JOHNSON: The morphine had been running for 5 minutes short of 2.5 hours?

LETBY: Yes.

JOHNSON: And yet the tube slipped again at 06.15?

LETBY: Yes.

JOHNSON: Yes, just after you'd been with her. Do you agree with that?

LETBY: I can't say that I was physically with [Baby K], no.

I don't have any recollection of being with [Baby K], no.

JOHNSON: Where had you got the notes from?

LETBY: That's different to being in -- physically touching [Baby K]. The notes are kept on a trolley at the end of the bedside, they're not physically with the baby. So I can't say that I was physically with [Baby K], touching [Baby K].

JOHNSON: Presumably, if you took the notes back, and there was some issue on the monitor you would have seen it, wouldn't you?

LETBY: If there was, yes.

JOHNSON: Yes. I'd like to move on to [Baby K]'s third desaturation before we come to the one involving Dr Jayaram when he says you saw you standing over her; okay?

LETBY: Okay.

JOHNSON: This was a desaturation at about 07.30 that morning, 07.25. Do you remember it?

LETBY: No.

JOHNSON: Do you remember saying to the jury in answer to your own counsel that you had nothing to do with this desaturation?

LETBY: Yes, I don't have any recollection of at all, no.

JOHNSON: Well, is that the same as having nothing to do with it?

LETBY: I can't recall any intervention at that point, no, so I can't say.

JOHNSON: Do you remember the evidence of [Nurse B] on 1 March about this?

LETBY: Yes.

JOHNSON: Do you remember the reason that she said her attention was drawn to this third desaturation of [Baby K]?

LETBY: Yes, she said I called to her from the nursery.

JOHNSON: Yes. What were you doing in nursery 1 at 07.30?

LETBY: I can't answer that. I don't have any recollection of that.

JOHNSON: What should you have been doing at 07.30 that morning?

LETBY: We would be preparing for handover.

JOHNSON: Where were the babies that you were handing over?

LETBY: Nursery 2, is it?

JOHNSON: Yes.

LETBY: Yes.

JOHNSON: So the question is: what were you doing in nursery 1?

LETBY: Well, I can't answer that. Staff go between the nurseries all the time.

JOHNSON: Yes, but why would you be -- I'm interested: it's all very well saying staff go between the nurseries all the time, but what were you doing in the nursery at that time?

LETBY: I can't answer that because I don't have a specific memory, so I can't answer that.

JOHNSON: Well, let's look at the neonatal review, please, which I think is still open in front of you. Can we look at page 7 of 9, please, to look at what you were doing in the half hour before [Baby K] desaturated for the third time.

In the bottom third or so of the page, can you see there's quite a lot of blue font?

LETBY: Yes.

JOHNSON: If you go to line 232, do you see that half an hour or 25 minutes or so before [Baby K] desaturated, you were attending to AR?

LETBY: Yes.

JOHNSON: And AR was one of your children in nursery 2?

LETBY: Yes.

JOHNSON: Just remind ourselves at page 2 of 9 of who your other child or other designated child was. It was GT; is that right?

LETBY: Yes.

JOHNSON: And we see that the last mention of GT before [Baby K]'s desaturation, which is shown at line 238, your last recorded contact with GT was at line 227 at 06.49 --

LETBY: Yes.

JOHNSON: -- where you were co-signing for medication that was given to her at that point?

LETBY: Yes.

JOHNSON: So there was no reason, was there, for you to have been in nursery 1 at 07.30?

LETBY: I can't answer that now because, as I've said, there's many reasons why staff would enter another nursery.

JOHNSON: Well, let's concentrate on the two children that you were designated to look after, please, GT and AR.

You've had access to their medical records, haven't you, as part of this process?

LETBY: Yes.

JOHNSON: Is there anything in their medical records that suggests there was something going on at 7.30 which would have necessitated you being in nursery 1?

LETBY: No.

JOHNSON: You've told us, and we've heard from many witnesses, that the handover begins at about 7.30.

LETBY: Yes.

JOHNSON: And so the question is, why were you in nursery 1?

LETBY: I can't answer that question.

JOHNSON: The answer, Lucy Letby, is that you were sabotaging [Baby K] yet again, weren't you?

LETBY: No.

JOHNSON: Why didn't you make a note of what happened at 07.30?

LETBY: I don't recall that happening.

JOHNSON: Sorry?

LETBY: I don't recall this event happening.

JOHNSON: No, but do you accept it happened?

LETBY: No, I have no memory, so I cannot say either way.

JOHNSON: Do you remember your colleague [Nurse B] giving evidence about it?

LETBY: Yes.

JOHNSON: And it was not disputed with her, was it, that this event had happened and that you had called her to nursery 1?

LETBY: No, but I can't categorically say whether that did or didn't happen. I have no memory.

JOHNSON: But she categorically says it did, didn't she, and you didn't dispute it?

LETBY: No.

JOHNSON: Do you dispute it in your defence statement?

LETBY: No, I say I have no memory.

JOHNSON: Were you trying to create the impression that [Baby K] habitually moved her own tube?

LETBY: No.

JOHNSON: Was [Baby K] a baby who more than once moved her own tube?

LETBY: I can't answer that.

JOHNSON: Why can't you answer it?

LETBY: Because I can't say how the tubes have dislodged if that's what's happened.

JOHNSON: Well, did you have a memory of [Baby K]'s tube moving more than once?

LETBY: From reading the notes, yes.

JOHNSON: Right. I don't quite understand the answer, that answer. What do you mean?

LETBY: I don't have an independent memory of the tube slipping, no. It's only through police interviews, being informed that it slipped several times and looking at the notes that I recalled that.

JOHNSON: I'm going to give you in a moment a rather fuller version of your interview, but if you could just get the interview bundle number 2, please, and go to the second of the [Baby K] interviews, so this is the interview of 12 June 2019. The extract that begins at [Initials of Baby K] 6. The time on 12 June was 09.51.

Just take as much time as you need to orientate yourself, but do you see at page 6 at the beginning of this interview, the police were asking you about Joanne Williams leaving the unit to update [Baby K]'s parents?

LETBY: Yes.

JOHNSON: We know that that was at 3.47 from the door swipe data, don't we? Do you remember that?

LETBY: Yes.

JOHNSON: Then on the next page, the police were asking about Dr Jayaram, which is something that I'm going to come to in a moment.

LETBY: Yes.

JOHNSON: Got that?

LETBY: Yes.

JOHNSON: Then at the bottom of the page the officer says to you:

"Dr Jayaram states that [Baby K]'s ET tube had become dislodged and there was no fault with the ventilator and the tube was replaced. He goes on to clarify that a tube can become dislodged if a baby is active and moving around."

Do you see that?

LETBY: Yes.

JOHNSON: Then over the page, please, on to page 8. They then start to ask you another question but you talk across them and say:

"I didn't dislodge her tube."

LETBY: Yes.

JOHNSON: You then say:

"I believe earlier in the shift her tube had slipped and she's already had her tube replaced earlier in the shift."

What were you talking about there?

LETBY: So there I was talking about a previous -- I'd got the dates wrong. I'm referring to an incident with Mel Taylor on a previous shift where the tube had slipped, because that was the notes that I had at that time.

JOHNSON: We're talking about [Baby K].

LETBY: Yes.

JOHNSON: [Baby K] was only in the Countess of Chester from about some time after 2 o'clock on the 17th.

LETBY: Yes. I think in one of my interviews, though, I was given the evidence of Mel Taylor; is that not right?

JOHNSON: I'm asking you.

LETBY: Well, I can't -- without reading through this now, I can't see... I'm not sure.

JOHNSON: Were you there trying to create the impression with the police that [Baby K] was a child who habitually displaced her own tube?

LETBY: No. But she had -- it had come out more than once, I believe, from the notes.

JOHNSON: Oh, it did, because on three occasions, we suggest, you deliberately displaced it --

LETBY: No, I did not.

JOHNSON: -- because you were trying to kill her.

LETBY: No.

JOHNSON: Do you dispute the evidence of [Nurse B] regarding that desaturation at 7.30 or thereabouts?

LETBY: Again, I don't think I can comment on whether that's the truth or not. I just have no memory of it. That's not for me to say whether she's accurate or not. I have no recollection.

JOHNSON: You're lying about that, aren't you?

LETBY: No.

JOHNSON: If you were there, as described by [Nurse B], would you normally have made a note?

LETBY: If I... If I was there, no, I can't -- no, not necessarily, no.

JOHNSON: Why did you make a note in the case of [Baby I] on 13 October then?

LETBY: In relation to what?

JOHNSON: The desaturation in the dark with Ashleigh Hudson.

LETBY: Because that was a life-threatening event that I'd come across.

JOHNSON: So was this.

LETBY: I then took over [Baby I]'s care.

JOHNSON: So this is for a child of 25 weeks, isn't it, 25 weeks' gestation, wouldn't you say?

LETBY: I can't answer that. I can't remember it.

JOHNSON: It was left for Mel Taylor to make a note, wasn't it?

LETBY: I don't recall.

JOHNSON: Well, I'll remind you. It's tile 166. If we go on to the document, please. Turn it sideways. Thank you.

I've got the wrong reference here. Let me just check.

(Pause)

MR MYERS: While we check the reference, can I just assist with one matter as a matter of record, my Lord?

MR JUSTICE GOSS: Yes.

MR MYERS: We're jumping around between the counts. On 13 October with [Baby I], just going to the notes quickly, care was actually transferred by Ashleigh Hudson or from Ashleigh Hudson to Ms Letby, hence Ms Letby made a note. She was just asked why she didn't make notes with [Baby K], but as I follow it care wasn't transferred in the same way. It might be a bit much to expect her to recall back to what the notes say on 13 October, so I'm just pointing that out now as to the reason for the note on the 13th with [Baby I].

MR JUSTICE GOSS: Thank you.

MR JOHNSON: Anyway, let's go to the desaturation at 03.50.

Okay?

LETBY: Okay.

JOHNSON: I'd like to start by going back to the neonatal review, please, which is underneath your interview bundle there.

So if we go to page 5 of 9, please. If we look at lines 115 and 116, do we see there that you were conducting observations of and feeding the child GT?

LETBY: Yes.

JOHNSON: And GT was in nursery 2?

LETBY: Yes.

JOHNSON: This was a feed of 55ml of milk?

LETBY: Yes.

JOHNSON: Which I think you've told us would take about 20 minutes or thereabouts?

LETBY: Approximately, yes. I think she was offered a bottle at that feed as well.

JOHNSON: Right. Do you have a memory of this?

LETBY: No. Not a direct memory, no, just reading from the charts.

JOHNSON: Right. I just want to see what you agree with and what you disagree with; okay?

LETBY: Okay.

JOHNSON: Do you agree that Joanne Williams was [Baby K]'s designated nurse on this shift?

LETBY: Yes.

JOHNSON: Do you agree that she, that is Joanne Williams, left [Baby K] at 03.47?

LETBY: I agree that she left her. I don't think I can say the specific minute that Jo left.

JOHNSON: Well, from memory perhaps not, but from the evidence -- I think it's tile 63, if we can go to that, please.

This is the swipe data. Do you remember you being referred to this --

LETBY: Yes.

JOHNSON: -- by your own counsel?

LETBY: Yes, I just couldn't recall the exact digits.

Thank you.

JOHNSON: It's all right, I'm not criticising you for not being able to remember a precise tile number or time even.

But you may remember this is the one where, when you were being asked questions about your own counsel, there was a mistake made about which way Joanne Williams was going.

LETBY: Yes.

JOHNSON: It was suggested to you that she was coming into the neonatal unit, whereas this shows she was leaving the unit.

LETBY: Yes.

JOHNSON: And so do you accept that she left the unit at that time?

LETBY: Yes.

JOHNSON: Do you accept that at that time Dr Jayaram was sitting at the nurses' station?

LETBY: Again, I can't answer that because I don't have any recollection. I can't recall Dr Jayaram's whereabouts.

JOHNSON: Okay. Well, let's look at the evidence. Tile 59, please. These are notes made by somebody in a different hospital; okay?

LETBY: Yes.

JOHNSON: As always, if you want to go to the originals, which are behind this, we can go to them. We've shown them already in the course of the evidence, but this was somebody from the transport team who was speaking to Dr Jayaram on the phone.

LETBY: Okay.

JOHNSON: Is there a phone at the nurses' station?

LETBY: There is, yes.

JOHNSON: And this is timed by that person 6 minutes before Joanne Williams left to go to speak to [Mother of Baby K].

LETBY: Yes.

JOHNSON: And so with that in mind, do you accept that Dr Jayaram was at the nurses' station?

LETBY: Again, I don't know how I can answer that because I can't -- that's reliant on his account. I have no recollection to say whether that did or didn't happen.

But if that is correct and he was sat at the nurses' station on at that specific phone then, yes.

JOHNSON: Just so we understand, how many phones are there that somebody from an external source could ring into the neonatal unit?

LETBY: There's four phones on the unit.

JOHNSON: Where are they?

LETBY: There's one in nursery 1 and then there's three outside on the nursing station.

JOHNSON: Right. So if the -- where do external calls usually come in, the nursing station or nursery 1?

LETBY: Usually the portable phone at the nursing station, but it's a portable phone so I can't 100% say that it was answered at the desk.

JOHNSON: No, but we know what he says about it anyway, don't we?

LETBY: Yes.

JOHNSON: Do you dispute that, given [Baby K]'s condition, her age, her extreme prematurity, that Joanne Williams would have asked one of her colleagues to babysit?

LETBY: Yes. A common practice would be to have somebody in the nursery, yes.

JOHNSON: Yes. Do you dispute that it was you?

LETBY: I have no memory of that.

JOHNSON: Well, I know, but do you dispute the evidence that it was you?

LETBY: I -- I don't know how I can answer that, sorry.

I don't... If I can't remember, I don't think I can say whether somebody else's recollection is right or not.

JOHNSON: How many babies of [Baby K]'s gestation had you seen before [Baby K] appeared in the neonatal unit?

LETBY: In the Countess of Chester?

JOHNSON: Yes.

LETBY: Very few, if any.

JOHNSON: Was she a memorable arrival?

LETBY: Yes.

JOHNSON: Do you remember her?

LETBY: No -- well, I remember the fact that we had a baby of that gestation on the unit, yes.

JOHNSON: Do you dispute that when Dr Jayaram walked into nursery 1, you were standing over [Baby K]?

LETBY: I have no recollection, so I can't confirm or deny.

JOHNSON: Do you dispute that when Dr Jayaram walked into nursery 1, [Baby K] was desaturating?

LETBY: Again, I can't comment. I have no memory.

JOHNSON: Do you dispute that the saturation monitor was down into the 80s?

LETBY: I have no memory.

JOHNSON: And the alarm was not sounding?

LETBY: Again, I don't think I can answer because I don't recall that.

JOHNSON: Do you dispute that the cause of [Baby K]'s desaturation was, again, a displaced ET tube?

LETBY: Again, I can't recall.

JOHNSON: Because there were three occasions when she did desaturate on that shift, weren't there?

LETBY: Yes.

JOHNSON: Yes. Each occasion involved a displaced ET tube; do you agree?

LETBY: If that's -- I can't recall exactly, but if that's the evidence then, yes.

JOHNSON: And the evidence is that on each occasion you were there, isn't it?

LETBY: I can't say that, no.

JOHNSON: The reason for those coincidences is because you were trying to kill [Baby K], weren't you?

LETBY: No.

JOHNSON: I'd like to -- given your earlier evidence that you weren't there when Dr Jayaram walked in on this occasion at about 03.50, I would like to give you a slightly different version of your interview with some bits that were taken out and put back in. If I can distribute these, please, my Lord. If I can start with the witness.

(Handed)

(Pause)

MR JUSTICE GOSS: You'll see where these go, behind 9, because they're marked 9A, B, C, D, E, so [Initials of Baby K] 9.

MR JOHNSON: They run sequentially on, my Lord, from -- it's the second interview.

MR JUSTICE GOSS: Yes.

MR JOHNSON: It's a fuller version, of course, of the second interview. If I can just give a bit of an explanation to introduce what we have here.

MR JUSTICE GOSS: Yes. Am I right in suggesting they go at the back of -- no, I'm wrong.

MR JOHNSON: The back of divider 2 --

MR JUSTICE GOSS: Behind [Initials of Baby K] 9?

MR JOHNSON: Yes.

MR JUSTICE GOSS: And you'll explain its relationship to [Initials of Baby K] 9 and its predecessor.

MR JOHNSON: Page 1 -- sorry, page [Initials of Baby K] 9 is -- well, the difference is, I think, it's a slightly smaller font, as the jury will see if they compare like with like, but the

important thing for you to understand, Lucy Letby, is that the differences between what we've already had and what we have here are now in italics. Okay?

LETBY: Yes.

JOHNSON: So looking at [Initials of Baby K] 9B, we can see that there are a couple of -- a question and answer that have been inserted four and five lines up from the bottom of the page.

LETBY: Yes.

JOHNSON: On 9C, there is no difference. 9D, it's the bottom third of the page that's come back in.

LETBY: Yes.

JOHNSON: And on 9E, a couple of answers and a question at the top of the page and then two answers and a question at the bottom of the page.

LETBY: Yes.

JOHNSON: It's actually the middle of the page but it's the end of the text on that page.

I'm going to suggest to you that in this interview, on 12 June 2019, the police were in effect taking you through Dr Jayaram's allegation that, when he went into nursery 1 at about 03.50 on the morning of 17 February, you were standing over [Baby K], who was desaturating.

Do you accept that, first of all?

LETBY: Yes.

JOHNSON: And you understood, didn't you, what the allegation was?

Didn't you?

LETBY: At this point?

JOHNSON: Yes.

LETBY: Um, yes.

JOHNSON: You understood that you had been arrested, so far as [Baby K] was concerned, on an allegation that you were the person that had moved the tube, putting it simply?

LETBY: Well, I don't think it was worded to me in that way.

I can't...

JOHNSON: It wasn't worded to you in that way but that was the essence of the allegation against you?

LETBY: Yes.

JOHNSON: All right. And this, of course, was almost exactly a year after you had first been arrested and questioned about [Baby K].

LETBY: Yes.

JOHNSON: To remind ourselves, if we just go back to divider 1, [Initials of Baby K] 1, we can see that you were questioned about the events concerning [Baby K] on 5 July 2018; is that right?

LETBY: Yes.

JOHNSON: Okay, so here we were 11 months and a week later, on 12 June 2019, and the police were asking you more questions about it, weren't they?

LETBY: Yes.

JOHNSON: So I don't want to spend a long time doing this, but can you go to -- if at any point you want to spend time just checking things, please do. At the top of [Initials of Baby K] 9B, so the second page, the police say to you:

"Do you agree that if there were any issues with [Baby K]'s tube that Joanne Williams would have remained with her?"

And you say yes.

LETBY: Yes.

JOHNSON: And then they put to you what Dr Jayaram was saying had happened --

LETBY: Yes.

JOHNSON: -- which is essentially the evidence that he gave to the jury earlier in the trial.

LETBY: Yes.

JOHNSON: He was at the nurses' station, he felt uncomfortable, he knew that no other members of staff were there and he went into nursery 1 --

LETBY: Yes.

JOHNSON: -- and he walked in. They asked whether you agree and you say, "I don't remember".

LETBY: Yes.

JOHNSON: Slightly further down, they say:

"If the saturations had dropped to the 80s and were still falling, would you expect the alarm to be activated?"

And you say yes.

LETBY: Yes.

JOHNSON: Then they say:

"Explain to me what you were doing, Lucy, when Dr Jayaram walked in?"

And you say:

"I don't recall."

LETBY: Yes.

JOHNSON: That wasn't true, was it?

LETBY: Yes, it is true.

JOHNSON: Page [Initials of Baby K] 9D, please. Almost exactly halfway down the page, the police put to you what Dr Jayaram said, that to get to that level of desaturation would take about 30 seconds.

LETBY: Yes.

JOHNSON: And you say you don't remember.

LETBY: Yes.

JOHNSON: You deny then that you dislodged the tube.

LETBY: Yes.

JOHNSON: And they then say to you:

"Is there a reason why he might be mistaken?"

Referring back to Dr Jayaram; is that right?

LETBY: Yes.

JOHNSON: Did you ever say to the police what you said to the jury, namely that this didn't happen?

LETBY: No.

JOHNSON: Why not?

LETBY: Because I can't say whether it did or it didn't. I have no memory of it, which is what I've said throughout.

JOHNSON: Well, I'm not sure it quite is what you've said throughout because in answer to questions from your counsel, and indeed to me when I started to ask you questions, you, I would suggest, were saying that it never happened. Did you make a mistake when you said that?

LETBY: No, I don't believe it did happen. I think -- but I have no direct memory of it.

JOHNSON: And it happened twice again on that shift, didn't it?

LETBY: What's that?

JOHNSON: The desaturation and your presence closely connected with those desaturations.

LETBY: Yes.

JOHNSON: Are you, just so the jury understands, suggesting that if a baby of [Baby K]'s gestation and condition had been desaturating, you would simply wait there to see if she self-corrected?

LETBY: If that's what I had seen, yes, I would wait, yes.

JOHNSON: You would wait?

LETBY: Yes.

JOHNSON: Despite the evidence, the agreed evidence, of the expert nurse Elizabeth Morgan --

LETBY: Yes.

JOHNSON: -- who said it is not normal nursing practice to do what you say you would have done?

LETBY: I disagree, but...

JOHNSON: Why do you disagree?

LETBY: Because I know common practice in the neonatal unit in Chester and Liverpool Women's is that you would wait a few seconds to see if the baby self-corrected, if they are not at a life-threatening level.

JOHNSON: What is a few seconds?

LETBY: Ten, 20.

JOHNSON: What about 30?

LETBY: I can't say. There's no definitive rule. I can't say.

JOHNSON: Thirty seconds?

LETBY: I can't say how long.

JOHNSON: You are lying, aren't you --

LETBY: No.

JOHNSON: -- because you tried to kill [Baby K]?

LETBY: No, I did not.

JOHNSON: Do you remember the evidence of Dr Smith, who re-intubated [Baby K] at 03.50?

LETBY: No, I don't, no.

JOHNSON: He told the jury that he had made no notes and had no recollection of there being any blood in her throat.

LETBY: Okay.

JOHNSON: Do you remember that?

LETBY: No.

JOHNSON: And yet, in the nursing note at T66 -- can we just look at that, please -- there is there mention of bloodstained oral secretions. About halfway down the page on the left --

LETBY: Yes.

JOHNSON: -- it says:

"Removed and re-intubated on second attempt by Registrar Smith with 2.5 ETT measuring 6.5 centimetres at lips. Air entry clear and equal. [Large] amount bloodstained oral secretions."

That's in the nursing note.

LETBY: Yes.

JOHNSON: You were the person who was actually helping with this re-intubation, weren't you?

LETBY: I can't say from memory.

JOHNSON: You were the person who signed, or one of the people who signed, for the morphine infusion at this time, weren't you?

LETBY: Yes, the morphine infusion commenced after intubation, yes.

JOHNSON: Yes, at 03.50?

LETBY: Yes.

JOHNSON: That's the time on the document, isn't it?

LETBY: Yes.

JOHNSON: Yes. Did you give that little detail to your friend Joanne Williams?

LETBY: The morphine detail?

JOHNSON: No, the bloodstained oral secretions.

LETBY: No. No, I don't think so, no.

JOHNSON: Was it designed to provide a reason for [Baby K]'s desaturation that was independent of the ET tube slipping?

LETBY: I have no recollection of saying anything about bloodstained oral secretions.

JOHNSON: Did you ever see [Baby K]'s parents?

LETBY: I can't recall.

JOHNSON: Did you ever meet them?

LETBY: I can't recall.

JOHNSON: Well, why were you searching for them on 20 April 2018, 2 years later?

LETBY: Because I have thought of babies on the unit over the years and I do look back on parents.

JOHNSON: Yes, but let's just examine that for a second, please, your recollection -- and you've repeated it many times this morning -- is you have no real memory of [Baby K].

LETBY: No, not in any detail, no.

JOHNSON: When you were giving answers to your own counsel, you said that you had a vague, unspecific memory of popping in to have a look at her because of her gestation --

LETBY: Yes.

JOHNSON: -- and that was it.

LETBY: Yes.

JOHNSON: Do you have a very good memory for names?

LETBY: Yes.

JOHNSON: Yes. Because her name didn't appear on a handover sheet, didn't it?

LETBY: [Baby K]'s name?

JOHNSON: Not one that you had anyway, did it?

LETBY: I can't say [Baby K]'s name was on there or not. I don't know.

JOHNSON: She wasn't on the handover sheet on the shift that you started into 17 February, was she --

LETBY: No.

JOHNSON: -- because she was born at 02.12 on the morning of the 17th?

LETBY: Yes.

JOHNSON: She was handed over to Mel Taylor; is that right?

LETBY: Yes.

JOHNSON: And you didn't have Mel Taylor's handover sheet, did you?

LETBY: No.

JOHNSON: No, and she was transferred out of the Countess of Chester shortly thereafter, wasn't she?

LETBY: Yes.

JOHNSON: How did you remember the name?

LETBY: I can't answer that.

JOHNSON: Oh, surely this is something you can answer because it is a feat of memory. You tell us how you remembered.

LETBY: I can't.

JOHNSON: Can't or won't?

LETBY: I can't.

JOHNSON: Is it because the true answer is you remembered her because you tried to kill her?

LETBY: No.

JOHNSON: What was the significance of 20 April 2018?

LETBY: I -- I can't -- I don't know.

JOHNSON: Do you remember the answer you gave to your counsel --

LETBY: No.

JOHNSON: -- on 16 May?

LETBY: No.

JOHNSON: "[You] still think of patients that you cared for."

When did you care for [Baby K]?

LETBY: I think that's taking it out of context. It doesn't have to be a baby that we're directly allocated to look after. We're all part of that care team that are on on that shift. I did play a part in [Baby K]'s care.

JOHNSON: Which part was that?

LETBY: The morphine.

JOHNSON: So a single morphine infusion signed at 03.50 was significant to fix her in your brain for 2 years?

LETBY: No, that's not what I'm saying.

JOHNSON: Yet you're saying you cannot remember standing over her at 03.50?

LETBY: No, I can't recall that, no.

JOHNSON: I'd like to move on now to the case of [Baby F]. If you haven't put the interviews away, ladies and gentlemen, it's probably a good time to do it.

I don't know where you put them, ladies and gentlemen, on Friday, but the defence statements, please. I'll start with that.

MR JUSTICE GOSS: I should imagine they're loose. I still have them treasury-tagged as a separate document.

You'll remember it well, I'm sure.

MR JOHNSON: We'll have to rewind a few pages back to [Baby F] because I've taken this one out of the chronological sequence.

MR JUSTICE GOSS: It starts at the bottom of page 10.

MR JOHNSON: Thank you.

You deal with this case as follows. Paragraph 79:

"I did nothing to hurt [Baby F]."

Then 80:

"I did not administer insulin to him or interfere with any of the total parenteral nutrition bags so as to make this happen. Because I know I didn't do this but I'm being blamed for it, I cannot accept that the material readings and measurements over the relevant period or blood analysis are necessarily accurate. If [Baby F] did receive insulin that he shouldn't have received, I did not do that."

Then 81:

"I wasn't the designated nurse for [Baby F] on the 4th to 5 August 2015."

82:

"I do not specifically remember hanging up the TPN bag at 00.25 on 5 August. I think I signed for the bag and [Nurse A] hung it."

Then 83:

"If you change a long line then you change the TPN bag."

Finally 84:

"The TPN fridge is locked. You have to get the key from the shift leader in order to gain access to the fridge."

Is that what's in that document?

LETBY: Yes.

JOHNSON: And is that true?

LETBY: Yes.

JOHNSON: Do you dispute the readings and measurements over the relevant period?

LETBY: No, I accept them now. At this point I didn't, but I do now, yes.

JOHNSON: Okay, that's fine. And by readings and measurements what are you talking about?

LETBY: The insulin level and the C-peptide level.

JOHNSON: The reason I asked you the question in that way is because there are a number of issues here. The principal issue, of course, is the one you have identified, which is the insulin and C-peptide.

LETBY: Yes.

JOHNSON: But there's another issue as well, isn't there, which is the blood sugar levels --

LETBY: Yes.

JOHNSON: -- of [Baby F] over the period with which we are concerned.

LETBY: Yes.

JOHNSON: So you've accepted the first point. Do you accept the blood sugar level readings?

LETBY: No, I think there may have been some discrepancies.

JOHNSON: Right. What are you saying -- we'll go to the specifics, but what are you saying in general terms are the discrepancies?

LETBY: That there were values that were different on lab results compared to results taken either on the unit or on the blood gas machine.

JOHNSON: Well, there is only a single lab result, isn't there?

LETBY: Um... I can't recall.

JOHNSON: Do you remember the evidence of Professor Hindmarsh, who told us about there is always a discrepancy between lab results and --

LETBY: Yes.

JOHNSON: -- blood gas readings because of the methodology of analysis?

LETBY: Yes.

JOHNSON: Because it's a spun sample, I don't off the top of my head remember the precise detail, but there tends to be about a 10 to 15% discrepancy.

LETBY: Yes, I wasn't aware of that until then.

JOHNSON: But putting that issue to one side, apart from that issue, are you disputing the readings of, in effect, the gas machine on the ward?

LETBY: No.

JOHNSON: No, all right. And just paragraph 82, I'd like to ask you a question about that. You say you don't specifically remember hanging the bag, but you think [Nurse A] hung it.

LETBY: Yes.

JOHNSON: It's probably me being a bit slow, but I don't understand how those two statements go together. You either remember it or you don't. So do you remember?

LETBY: No.

JOHNSON: Well, why do you think [Nurse A] --

LETBY: Because I signed the bag with [Nurse A]. [Nurse A] was [Baby F]'s allocated nurse. I don't recall hanging up the bag, so to me the only other person who could have done is [Nurse A].

JOHNSON: So because you don't remember doing it, it must have been her?

LETBY: I'm not saying it must have been, but I think, yes.

JOHNSON: You'll remember, I'm sure, that on 18 May I asked you whether you agreed that [Baby F] was poisoned with insulin.

LETBY: Yes.

JOHNSON: Remember that question?

LETBY: Yes.

JOHNSON: And you did agree?

LETBY: Yes.

JOHNSON: As you agreed that somebody must have given him insulin unlawfully?

LETBY: Yes.

JOHNSON: Do you still agree?

LETBY: Yes.

JOHNSON: Another question I asked you was whether somebody had targeted him specifically; do you remember that?

LETBY: I don't recall, but...

JOHNSON: Right. Well, I am suggesting that somebody did target [Baby F] specifically. Do you agree or not?

LETBY: I can't answer that.

JOHNSON: You did agree that a mistaken administration of insulin wasn't an option here. Do you still agree that?

LETBY: Um, from the neonatal unit, yes, I don't think that could have happened on the unit.

JOHNSON: Do you accept that somebody put insulin into the TPN bag or TPN bags that was or were connected to [Baby F]?

LETBY: I don't think I can say exactly what was in what bag, but I accept that he was given insulin at some point, yes.

JOHNSON: But the insulin has to have come from the bag, doesn't it, given the evidence we've heard? Do you accept that?

LETBY: Yes, if that's the evidence, yes.

JOHNSON: Yes. When you were arrested, you didn't know about C-peptide, did you?

LETBY: No.

JOHNSON: You didn't know that if a blood sample was taken from a child who was being given exogenous insulin, that fact could be proved by the disconnect in the ratio between the level of insulin and C-peptide. You didn't know that, did you?

LETBY: No. I didn't know anything about the C-peptide results, no.

JOHNSON: But had you ever heard of C-peptide?

LETBY: No, no.

JOHNSON: In answer to questions from your own counsel, or under questioning from your own counsel, you were asked a series of questions about the insulin and TPN being in the same fridge in what is a busy room.

LETBY: Yes.

JOHNSON: But whether or not the room was busy, somebody put insulin into that bag, didn't they?

LETBY: If it's agreed that insulin was in that bag, I can't say where it went in, whether it was on the unit or elsewhere. I can't answer that.

MR JUSTICE GOSS: I'm sorry, elsewhere? Where else?

LETBY: The bag is made by -- the bag comes from the CIVAS unit.

MR JUSTICE GOSS: So it could have been done by somebody in the CIVAS unit, is that what you're saying?

LETBY: I can't say, but I --

MR MYERS: She say she doesn't -- she cannot say where it went, wherever it went in, if it went in.

MR JUSTICE GOSS: But you're positing that as a possibility then?

LETBY: Yes.

MR JUSTICE GOSS: All right.

LETBY: He's asking if it was put in on the unit. I can't answer that, but potentially the bag has come from another area and I can't answer what happened in that other area.

MR JOHNSON: So that you can bear this in mind as we go along, this is one of the reasons that I'm dealing with [Baby F] and [Baby L] together. All right?

LETBY: Yes.

JOHNSON: Because I'm going to suggest to you that the insulin that went into [Baby L]'s dextrose bag definitely went in on the neonatal unit. All right?

LETBY: Okay.

JOHNSON: We'll come to the reasons for that in due course.

The bag was changed, wasn't it, at midday on 5 August, after you had gone off duty? Do you remember that?

LETBY: Yes.

JOHNSON: And so given that the evidence shows that insulin continued to be administered to [Baby F], either the giving set wasn't changed or a second bag was contaminated?

LETBY: Yes.

JOHNSON: Do you accept that?

LETBY: Yes.

JOHNSON: I'm going to come to the population distribution now, please, which is at tiles 100 and 101. 101 is the best, please.

Here we see, beginning with nursery 2, that you were in nursery 2. [Baby F] was the designated charge of your friend [Nurse A]?

LETBY: Yes.

JOHNSON: You were with a baby, ME?

LETBY: Yes.

JOHNSON: In nursery 3 there was, so far as we can tell, a single child, CE, and two children in nursery 4 being looked after by Valerie Thomas.

LETBY: Yes.

JOHNSON: There is an anomaly in the sense that there were two other children in the unit that day.

LETBY: Yes.

JOHNSON: One was JE, who was being looked after by Cheryl Cuthbertson-Taylor, who had a baby in nursery 3.

LETBY: Yes.

JOHNSON: From a practical point of view, was it common for one nurse to have a baby in nursery 1 and a single baby in nursery 3 as well?

LETBY: Sorry, can you say that again?

JOHNSON: Yes. I'm going to suggest to you that normal practice would suggest that because CE was in nursery 3 with Cheryl Cuthbertson-Taylor, the overwhelming likelihood is that so was the child JE.

LETBY: It's likely, but it's not -- I can't ever say definitively.

JOHNSON: No, of course not. Well, there was certainly no shortage of space in nursery 3, was there, on that information?

LETBY: No.

JOHNSON: All right. That leaves one other child, EJ, who was being looked after by Sophie Ellis, who, I think on 23 November, told us that she couldn't remember where he was.

LETBY: Okay.

JOHNSON: But insofar as we can determine the locations of these children, the child with whom we are principally concerned, [Baby F], was in the same room as you?

LETBY: Yes.

JOHNSON: Are you suggesting that staffing levels have in any way contributed to the poisoning of [Baby F]?

LETBY: I can't say how the insulin got there. I can't answer that.

JOHNSON: But it's nothing to do with staffing levels, is it?

LETBY: I don't know how the insulin got there, so I can't possibly answer how it might have happened or why.

JOHNSON: Well, somebody has -- hasn't somebody deliberately put insulin into the bag?

LETBY: If the evidence is accepted it's gone in, yes.

JOHNSON: And that can't have anything to do with staffing levels, can it?

LETBY: Again, I don't think I can answer that because I can't -- I don't know where that insulin came from or where it was put in to answer that.

JOHNSON: As you wish.

LETBY: Whether it was on the unit or elsewhere, I can't tell you that.

JOHNSON: When you were interviewed by the police, you were very keen to know whether they had access to the TPN bag, weren't you?

LETBY: Yes.

JOHNSON: Why?

LETBY: Because I was being accused of placing insulin in the bag and I thought that if the doctors had raised an issue and these levels were so abnormal that somebody would have thought to check the fluids, which is what we do routinely. As we've seen in other cases, we will keep the fluid bag if there were any concerns.

JOHNSON: But even if an analysis showed insulin was in the bag, that wouldn't prove that you had put it in the bag, would it?

LETBY: No -- I don't understand what you're asking me.

JOHNSON: Well, I think you've just --

LETBY: I wanted them to check the bag, yes. I felt that would have been standard practice. If, at the time, the doctors had concerns about the results, then they would have checked the fluids or made a point of looking and reflecting on the fluids.

JOHNSON: You wondered whether there was an issue with something else, didn't you?

LETBY: No.

JOHNSON: No? Can we look at your interview, please.

LETBY: Where is that, please?

JOHNSON: It's in the first bundle of interview transcripts, so interview bundle 1. Just to refresh our memories, particularly as we're jumping around in the chronology, the first interview took place on 10 June 2019.

Do you see that?

LETBY: Yes.

JOHNSON: So this was a -- it's behind divider 1, [Initials of Baby F]. You'll remember that was a case, we were told, that was only referred to the police because of what had happened to [Baby E]. Do you remember? That's why the first interview is a year later.

LETBY: Oh, okay, yes.

JOHNSON: So at your first arrest, 2018, you weren't asked any questions about [Baby F] or [Baby L].

LETBY: No.

JOHNSON: Do you remember that? And the reason was, as Dr Evans, I think, told us, he was then sent sibling cases from what he had identified as being suspicious cases.

LETBY: Yes.

JOHNSON: So that's the -- lest anyone has forgotten, that is the reason why you weren't interviewed the first time.

If you go to page 16, please, so [Initials of Baby F] 16, midway down the page you ask a question:

"Can I ask a question about this in terms of the bags and everything? I'm assuming they were -- they haven't been kept or checked, you know, post-event."

You knew very well, didn't you, that the bags hadn't been kept?

LETBY: No, I didn't know whether the bags had been kept.

JOHNSON: Did you think it was likely they would have been kept?

LETBY: Knowing that I kept the bag of fluids for [Baby A] and other babies, I felt that if the doctors had raised a concern at that time then, yes, potentially, the bags would have been checked, yes.

JOHNSON: Well, let's see what you were saying in this interview back in 2019. It's towards the bottom of the page:

"Is it likely the bags would be kept?"

Five lines up from the bottom. And what was your answer then?

LETBY: "No."

JOHNSON: To be fair to you, at the bottom you are then asked:

"You have asked the question, so are there cases when they might be?"

And you say:

"If there's a baby there's been a concern about, we would keep the bag, usually ask someone to check that bag or check the pump."

But you knew, didn't you, that no concern had been expressed at the time --

LETBY: No, I did not.

JOHNSON: -- about the bag. Didn't you?

LETBY: I didn't know anything at this point, at this interview, no.

JOHNSON: Do you remember me asking you a few minutes ago about whether you were thinking there was another way out for you, whether there was some other issue with the bag or an issue with something else?

LETBY: Yes.

JOHNSON: And you said no.

LETBY: Yes.

JOHNSON: Just look at the bottom of page [Initials of Baby F] 17, please. If you look at the 3100 number, counting up from the bottom of the page, three of those, question:

"Okay, is there a reason why you've asked that question? What's going through your mind?"

Asks the police officer. And what did you answer?

LETBY: "When something's happened in that time -- you are asking me if I have given him insulin and I'm wondering if there's an issue with something else."

JOHNSON: What was on your mind at that time? What was that something else?

LETBY: That the insulin had come from somewhere else other than the unit.

JOHNSON: How would the presence or absence of the insulin in the bag have assisted with the question of whether or not the insulin came from the unit or from somewhere else?

LETBY: It wasn't. But at this time I don't think it was suggested it was in the bag. We didn't know where -- we didn't know where the insulin had come from.

JOHNSON: No. You did, though, didn't you --

LETBY: No, I did not.

JOHNSON: -- because you put it there?

LETBY: No.

MR JOHNSON: My Lord, that's probably a good time for a break. I'm not going to finish this case in the next 5 minutes.

MR JUSTICE GOSS: Thank you. A 15-minute break, please.

(11.43 am)

(A short break)

(11.58 am)

MR JOHNSON: Do you remember telling the police in interview that you were not aware of there being a concern of [Baby F] having blood sugar problems?

LETBY: No.

JOHNSON: You don't remember saying that?

LETBY: At police interview?

JOHNSON: Yes.

LETBY: No.

JOHNSON: If we go back a couple of pages from where we were, do you see [Initials of Baby F] 15?

LETBY: Yes.

JOHNSON: Almost exactly halfway down:

"Were you aware, Lucy, as to the concerns for [Baby F] regarding his blood sugar levels at the time?"

LETBY: Okay.

JOHNSON: And you say, "No".

LETBY: Yes.

JOHNSON: You were aware though, weren't you, at the time?

LETBY: No.

JOHNSON: Can we go to the sequence, please, tile 222? This is after you've just gone off duty.

LETBY: Yes.

JOHNSON: Did you hear what [Baby F]'s sugar was at 8?

LETBY: Yes.

JOHNSON: Do you still say you were not aware?

LETBY: No, I was not aware in this interview in 2019 about blood sugars, no.

JOHNSON: You were aware at the time of the concern for [Baby F]'s blood sugars, weren't you?

LETBY: Yes, at the time because I was there.

JOHNSON: And you were telling your friend, [Nurse A], at 224, "1.8".

LETBY: Yes.

JOHNSON: And her response was, "Shit", if we look at the next.

Had you ever seen anything like this before?

LETBY: Babies with low sugars?

JOHNSON: Yes, but being given all this dextrose and still having low blood sugars.

LETBY: Yes.

JOHNSON: You had?

LETBY: Yes.

JOHNSON: You were trying to put it down to natural causes, weren't you?

LETBY: I don't think I was trying to provide an explanation, I was just stating what the levels were.

JOHNSON: Let's look at tile 341. What does that say?

LETBY: "Wonder if he has an endocrine problem then. Hope they can get to the bottom of it."

JOHNSON: What did you mean by an endocrine problem?

LETBY: The babies that I've previously seen needing the levels of glucose that [Baby F] was needing were babies that had endocrine problems, hyperinsulinaemia.

JOHNSON: Does that mean natural causes?

LETBY: Sorry?

JOHNSON: Does that mean natural causes?

LETBY: Yes.

JOHNSON: So you were trying to suggest natural causes, weren't you?

LETBY: Yes, the only other babies I had seen like that had a condition, yes.

JOHNSON: And you didn't know about C-peptide, did you?

LETBY: No.

JOHNSON: Do you accept that the TPN bags are not secure?

LETBY: No, I don't -- but the fridge is locked. I don't -- at what point? I... I think the bags are secure, yes, in the fridge.

JOHNSON: Secure in the sense they're under lock and key; is that what you mean?

LETBY: Yes.

JOHNSON: They're not secure from someone with the key who wants to tamper with them, though, are they?

LETBY: No.

JOHNSON: Do you remember the evidence of Mr Allen, Ian Allen, on 29 November?

LETBY: Not in full detail, no.

JOHNSON: Do you remember him demonstrating how you can take the supposedly tamperproof cap off a bag of TPN --

LETBY: Yes.

JOHNSON: -- and inject insulin into the bag?

LETBY: Yes.

JOHNSON: And the tiny amount of insulin that's required to produce results like this?

LETBY: Yes, but the bags in the fridge are sealed so you'd have to break the seal to do that.

JOHNSON: Yes. You either do it as the bag is being put up -- and who was putting up the bag, just remind us?

LETBY: Myself and [Nurse A].

JOHNSON: Yes. Or you do it once the bag is up and you were there, weren't you?

LETBY: I was there when the bag was hung, yes.

JOHNSON: Yes. And if we look at the staff levels -- sorry, the staffing arrangements for this shift and the shift we're about to come on to with [Baby L], only two people are common to each shift, aren't they?

LETBY: Yes.

JOHNSON: One is Belinda Simcock and the other is?

LETBY: Myself.

JOHNSON: Yes. So because the bags are in a seal, as you've just described, would that prevent someone from the previous shift having put insulin into the TPN bag without leaving a trace?

LETBY: I don't think I can answer that because I have never put insulin into a bag, so I can't say how it would or wouldn't be done.

JOHNSON: No, but in telling us about the security of these bags you referred to the seal. You describe to us what that seal is, please.

LETBY: So the TPN comes within a cellophane bag.

JOHNSON: How do you get into the cellophane bag?

LETBY: You would tear it open --

JOHNSON: Yes.

LETBY: -- at the time of drawing up the fluids, yes.

JOHNSON: Yes. It may be on your behalf the jury would be invited to consider whether or not someone on the previous shift put insulin into this bag, which came up at 4 pm.

LETBY: I can't answer that.

JOHNSON: Well, yes, but let's -- you can help us with the likelihood of it though by just answering a few questions, if you would.

First of all, do you agree with the evidence that we heard that these bags come up from the pharmacy at about 4 pm?

LETBY: Yes.

JOHNSON: So this was a bag that, if it had not been tampered with in the pharmacy, must have been tampered with some time

between about 4 pm on 4 August and about 01.00 hours on the morning of 5 August.

LETBY: Yes, if it was his prescribed bag, yes. If it was a generic bag, no, but prescribed, yes.

JOHNSON: This was a prescribed bag, wasn't it?

LETBY: Okay.

JOHNSON: That's the evidence.

LETBY: Okay.

JOHNSON: The replacement bag was a generic bag.

LETBY: Okay.

JOHNSON: So let's just think about this. A prescribed bag came up at 4 pm. How robust is the cellophane covering for the bag that has to be torn open to get access to the bag?

LETBY: It's cellophane. You would have to rip it.

JOHNSON: Is it completely sealed?

LETBY: Yes.

JOHNSON: Okay. So you saw Mr Allen demonstrate to us how the supposed tamperproof cap isn't a tamperproof cap at all, didn't you?

LETBY: Yes.

JOHNSON: So in order to get insulin into the bag once it's come up to the ward, if it's still in the cellophane wrapper you have to get somehow through the -- you have to get the insulin through the cellophane wrapper?

LETBY: Yes.

JOHNSON: You have to get the cap off the bag, still in the cellophane wrapper; is that right?

LETBY: If it was put in through that port, yes.

JOHNSON: Yes.

LETBY: I don't know. The bag was (overspeaking) --

JOHNSON: I know you don't know, but the jury may want to consider whether this is possible, you see. You then have to get a needle through the bag and through the tampered cap?

LETBY: Yes.

JOHNSON: Inject insulin into the bag?

LETBY: Yes.

JOHNSON: Replace the what was thought to be a tamperproof cover?

LETBY: Yes.

JOHNSON: And do all that without leaving a trace and without leaving a trace on the cellophane wrapper?

LETBY: Yes, if that's how it went in, yes.

JOHNSON: Yes. Whereas the other way of doing it, if it came up to the ward without insulin in it, is to do it once you've removed the cellophane wrapper; is that right?

LETBY: Yes.

JOHNSON: Which is something that, given the timings that we're talking about, only a person on your shift could have done?

LETBY: Yes.

JOHNSON: Yes. And there are very few candidates for that, aren't there?

LETBY: Yes.

JOHNSON: Why would you not put insulin in one of these bags?

LETBY: Why would I not put insulin?

JOHNSON: Yes.

LETBY: Because that's against all practice. TPN does not have insulin in it. We do not add insulin to anything unprescribed.

JOHNSON: It is highly dangerous, isn't it?

LETBY: Yes, yes.

JOHNSON: Life-threatening to a child of this age?

LETBY: Yes.

JOHNSON: And it is something that would never cross the minds of the medical staff that someone had done that, isn't it?

Well, did it cross your mind?

LETBY: At the time, no, no.

JOHNSON: All right. Let's go to the tile that -- T151, please.

This is the intensive care fluid chart, isn't it, filled in by your friend [Nurse A]; do you see that?

LETBY: Yes.

JOHNSON: There is an entry at 01.00 hours, isn't there? Can you see that?

LETBY: On the right-hand side, yes.

JOHNSON: Yes. It says "NG asp/vomit" and there are four pluses here.

LETBY: Yes.

JOHNSON: I don't think we've seen four elsewhere, but here we do.

LETBY: Yes.

JOHNSON: That, we heard in evidence, was a large vomit by [Baby F].

LETBY: Okay.

JOHNSON: And we also heard from Professor Peter Hindmarsh that vomiting is a classic symptom of low blood sugar.

LETBY: Okay.

JOHNSON: Do you remember that?

LETBY: I don't, but I accept that if that's what he said.

JOHNSON: Yes. That vomit was because by this stage -- do you remember what Professor Hindmarsh said about the half-life of insulin?

LETBY: No.

JOHNSON: That if it's in an infusion, because of the rate it's eliminated from the body, you get to the steady state in an infusion after about 25 minutes. Okay?

LETBY: Okay.

JOHNSON: Do you remember that now?

LETBY: Yes.

JOHNSON: And of course this is about, if we're looking at strict timings, it's 35 minutes after -- that's assuming that the 1 o'clock is bang on 1 o'clock. It's 35 minutes after this infusion was started by you and your friend [Nurse A], isn't it?

LETBY: Yes.

JOHNSON: Do you accept that insulin was either in the bag when it was hung or it was put into the bag shortly after it was hung?

LETBY: Yes.

JOHNSON: So do you accept that [Baby F] was poisoned deliberately?

LETBY: I can't answer that because I don't know how the insulin got there or who put it there or why. I can't answer that.

JOHNSON: I'll come back in due course to your searches on various parents in this case, but [Mother of Babies E & F] in particular was somebody you were searching for all the time, isn't she?

LETBY: Yes, frequently, yes.

JOHNSON: Why was that?

LETBY: [Mother of Babies E & F] was somebody who was often on my mind. [Baby E]'s death did stand out to me and I often thought of them as a family. I got on well with [Mother of Babies E & F] at the time.

JOHNSON: What about [Baby F]?

LETBY: I wanted to see how [Baby F] was doing.

JOHNSON: What had happened with [Baby F] that made you think of him?

LETBY: Because I got to know the family and obviously he was the surviving twin of [Baby E].

JOHNSON: Right. I'm going to move on to [Baby L] and see where the parallels are between what happened to him and what we've just established happened to [Baby F].

Okay?

LETBY: Okay.

JOHNSON: I'd like to start, as I tend to, with your defence statement, so we're going to have to scroll forward to take up the chronology at the correct place after [Baby K], please. This is paragraph 136 onwards.

It will be page 17 of the version you've got. It may be slightly different in the version the jury have because of the editing process.

You begin by saying that:

"[You] did nothing to harm either [Baby L] or [Baby M]. After the twins had been born on 8 April, [Baby L] and [Baby M] were put into nursery 1. Amy Davies and I took them to the nursery and we looked after them on their first day. I looked after [Baby L] and Amy looked after [Baby M]."

Then at 138 you say that [Baby M] wasn't in a proper space. At 139, I'm concentrating on [Baby L] for now:

"[Baby L] had low blood glucose levels. There was a conversation with Dr Bhowmik about why we were starting [Baby L] on fluids straightaway rather than doing a feed. Normally if the sugars are low you feed a baby and move on to fluids if they remain low. Starting with fluids is against that policy and it means a line is put in when it may not be necessary. Once Dr Bhowmik explained why this was the reasonable course to take, we proceeded as she instructed us."

And this was what -- is this the hypoglycaemic pathway, is that a reference to that?

LETBY: Yes.

JOHNSON: And this is an example, isn't it, of evidence that we've heard that you were more than willing to challenge doctors if you thought that they weren't doing the right thing?

LETBY: Yes, when they've deviated from a policy, yes.

JOHNSON: Yes:

"We put the bag up at 11 am on 8 April and the infusion started at midday. I wasn't due to be working on 9 April and I was asked to do the extra shift because the unit was so busy and we needed to get the skill mix right. We were exceptionally busy. I think I may have had three babies to look after."

As a matter of fact, as we will see, you had two.

LETBY: Yes.

JOHNSON: Is that right?

142:

"From the medical notes, the second bag of 10% dextrose was put up for [Baby L] at midday on 9 April. I did this with Mary Griffith. As the blood sugar levels remained low, a bolus was signed for and provided by Mary and me at 15.40."

And just to put that into chronological context, that's shortly before [Baby M] collapsed, isn't it?

LETBY: Yes.

JOHNSON: "I cannot be sure precisely who administered the bolus.

A new bag of 12.5% dextrose was signed for and given by Belinda Simcock and Ashleigh Hudson at 16.30. At 19.00 a new bag was signed for and given by Belinda Simcock and Mary Griffith."

And all this information relating to what was being done and when is, if you want to have it open, in jury bundle 1, so not the interview bundles but the other pair of bundles, number 1. It's the one with the big blue 1 on the back if that helps the gentleman who's helping you. Thank you.

If you look, if it's been done right, it should be behind divider 6. Have you got a table there that runs to three pages?

LETBY: Yes.

JOHNSON: So all this information is in there if you need to refer to it.

Then at 143:

"[Baby M] collapsed as Mary and I were making up fluids for [Baby L]. His alarm went off, so we went over to him had he was

apnoeic. When I look back, I am not sure if he wasn't breathing or whether he had had a desaturation."

You then talk about [Baby M], so I'm going to skate over that for now because we're concentrating on [Baby L], and to 150, please:

"I did not administer insulin to [Baby L]. I don't understand why [Baby L]'s insulin test results were at the level they were. Also, I don't understand how the results can have been so abnormal and yet there was no immediate investigation into this, therefore I am unable to accept the accuracy of these tests."

Do you now accept the accuracy of the tests?

LETBY: Yes.

JOHNSON: "If insulin entered [Baby L] via one or more of the bags that were used, I am not responsible for that."

Okay?

LETBY: Yes.

JOHNSON: So that's what you say in the defence statement. Save for the correction in relation to your acceptance of the test results, is there anything else in that information that is now, on review, incorrect?

LETBY: No, I don't think so, no.

JOHNSON: Right. So I just want to put [Baby L] into a bit of context very briefly. Born on 8 April 2016; is that right?

LETBY: Yes.

JOHNSON: As you said in the defence statement, you and Amy looked after one twin each?

LETBY: Yes.

JOHNSON: You had [Baby L]?

LETBY: Yes.

JOHNSON: Amy Davies is a band 6, isn't she?

LETBY: That's right.

JOHNSON: You identified straightaway that the hypoglycaemic pathway was not being followed?

LETBY: Yes.

JOHNSON: And I am going to suggest to you that that was one of the reasons why you decided to attack [Baby L].

LETBY: No.

JOHNSON: A departure from the pathway has nothing to do with somebody putting insulin into dextrose, though, does it?

LETBY: No.

JOHNSON: You told us, I think, that the dextrose bags are stored in nursery 1.

LETBY: Yes.

JOHNSON: The insulin in the locked fridge in the room where the locked fridge is?

LETBY: Yes.

JOHNSON: But the fact that they're stored in different places doesn't help us, does it, because somebody put insulin into the dextrose?

LETBY: Well, I -- I can't answer that.

JOHNSON: Well, do you accept from the evidence that somebody put insulin into the dextrose?

LETBY: Yes.

JOHNSON: And do you accept that there is no legitimate reason for putting insulin into dextrose?

LETBY: Yes.

JOHNSON: That it is highly dangerous?

LETBY: Yes.

JOHNSON: And that the blood -- the results of the blood test prove that somebody put insulin into [Baby L]'s dextrose?

LETBY: Yes.

JOHNSON: I want to see if you accept any of these propositions.

There was insulin in at least one bag of dextrose that was attached to [Baby L]?

LETBY: I'm... I'm not sure without looking.

JOHNSON: Well, at least one. We'll come to the precise number later perhaps, but given that --

LETBY: Yes.

JOHNSON: -- you have just accepted that there was -- well, do you accept it was in a bag of dextrose at any point?

LETBY: Yes, if that's the evidence, yes.

JOHNSON: Yes, absolutely. Do you accept that the first time it was done was some time between midnight and 9.30 am on 9 April?

LETBY: I don't think I can answer that. I'm just reliant on the expert opinion, I don't know.

JOHNSON: Well, it is, but it does rely on the expert, but of course you have said, "If I accept it, that's the evidence", in answer to a number of questions, so let's just look -- remind ourselves of the evidence looking at the chart that's there.

8 April 2016 was a long day so far as you were concerned?

LETBY: Yes.

JOHNSON: We can see the blood sugar reading in the third column between 10.58 and midnight, can't we?

LETBY: Yes.

JOHNSON: And we can see the gradual -- if you go to column 5, the dextrose rates don't change -- or "other event" column, we can see a gradual decrease in the amount of dextrose being given to [Baby L].

LETBY: Yes.

JOHNSON: And a gradual increase, albeit with some anomalies along the way, in the blood sugar reading?

LETBY: Yes.

JOHNSON: It's not a linear progression, it's not a straight line, but we go from 1.9 at the beginning of the day at 10.58 to 3.6 at the end of the day.

LETBY: Yes.

JOHNSON: Do you remember Professor Hindmarsh saying of the reading at 24.00 hours, at midnight -- I'm quoting him now:

"One could relax seeing that evolve."

In other words, the progression in the blood sugar.

LETBY: Yes, that's normal levels, yes.

JOHNSON: Exactly. And on this particular night shift of the 8th into the 9th, albeit she was a bank nurse so far as the Countess of Chester was concerned, the nurse was Tracey Jones --

LETBY: Yes.

JOHNSON: -- who we heard is a band 8A.

LETBY: Yes, she is, yes.

JOHNSON: That's a very, very senior nurse, isn't it?

LETBY: Yes.

JOHNSON: Yes.

LETBY: I don't know -- was she that at that point though?

I can't comment on that, I don't know.

JOHNSON: Well, my recollection is that's what she said --

LETBY: Okay.

JOHNSON: -- but it may not matter. We can see, by looking at the chart that you have there, that that was the final blood sugar measurement taken until 10.00 hours the following morning.

LETBY: Yes.

JOHNSON: And this is where we come back to a point that we've just dealt with in the context of [Baby F] about the half-life of insulin. Do you remember the 25-minute point?

LETBY: Yes.

JOHNSON: It's that point that led Professor Hindmarsh to say that by 9.30 on the morning of 9 April, somebody had put insulin into that dextrose bag.

LETBY: Okay.

JOHNSON: Do you accept that as a proposition, first of all?

LETBY: Yes, if that's what he said, yes.

JOHNSON: Yes. So no insulin in the bag up to midnight. Insulin in the bag some time at or just before 9.30 that morning.

LETBY: On his evidence, yes.

JOHNSON: On his evidence.

MR MYERS: His evidence actually was between midnight and before 9.30.

MR JOHNSON: That's what I said.

MR MYERS: You said "at or just before 9.30".

MR JUSTICE GOSS: The bracket is a lot wider. The last reading is at midnight, 24.00.

MR JOHNSON: That's certainly what I meant and if I said anything different...

MR JUSTICE GOSS: Anyway, because no one knows what the readings were between then and --

MR JOHNSON: Of course. I'm sorry if I've made a mistake about that. That's what I intended to say even if I didn't quite get it out right.

MR JUSTICE GOSS: Because there were no readings. You should have said, "We don't know what the levels were because there were no readings".

MR JOHNSON: All right.

I'd like to look at who was where and who was doing what at about 9.30, okay, that morning?

LETBY: Yes.

JOHNSON: The first point then -- do you remember me suggesting to you that this was a targeted attack against [Baby M]?

LETBY: [Baby L], do you mean?

JOHNSON: [Baby L], quite right. Against [Baby L].

LETBY: Yes.

JOHNSON: Just so you understand, the reason I am suggesting it is a targeted attack is that the same bag was hanging from midday on the 8th to midday on the 9th. Do you see that in the table?

LETBY: Yes.

JOHNSON: It didn't have insulin at the beginning -- on the 8th.

LETBY: No.

JOHNSON: But it had insulin in it on the 9th.

LETBY: Yes.

JOHNSON: Yes. So it follows, doesn't it, from that that the insulin was put up -- put into the bag whilst it was hanging, doesn't it?

LETBY: Um... I don't know.

JOHNSON: Well, let's just take a step back. We've dealt with the readings on 8 April; yes?

LETBY: Yes.

JOHNSON: They show that there was a naturally resolving hypoglycaemia that was resolved by the administration of the dextrose.

LETBY: Yes.

JOHNSON: So it had resolved by midnight?

LETBY: Yes.

JOHNSON: But then there was an exceptionally low and series of low blood sugar readings from 10.00 hours on the 9th.

LETBY: Yes.

JOHNSON: It was the same bag that was connected to [Baby L] from midday to midday?

LETBY: Right.

JOHNSON: So it follows, doesn't it, that somebody has put insulin into the bag whilst it is hanging?

LETBY: If that's what the experts suggest, yes.

JOHNSON: Well, it is what the experts suggest.

LETBY: Okay.

JOHNSON: Therefore it follows, doesn't it, that it is a targeted attack? The victim has been selected.

LETBY: Well, I can't answer that.

JOHNSON: All right. Somebody has injected insulin into a bag that is connected to a specific child?

LETBY: Yes, if that's the evidence, yes.

JOHNSON: Yes. Well, it is the evidence just as [Baby F] got, on one view of the evidence, two separate bags that were contaminated with insulin, TPN bags in his case.

LETBY: Yes.

JOHNSON: So let's look at the population distribution for this shift, first of all, please. It's tile 88, please.

Before I start this exercise, can you just put the interviews away because it may help you to concentrate on the things that we need to concentrate on. Keep that open but move on to [Babies L & M], please.

If we go to -- it's page 2 of 12 in the neonatal review, which shows the same information that we have at tile 88 on the screen. Okay?

LETBY: Yes.

JOHNSON: This shows four babies in nursery 1?

LETBY: Yes.

JOHNSON: The [Surname of Babies L & M] brothers being cared for by your friend Mary Griffith?

LETBY: Yes.

JOHNSON: You with GT and TSB?

LETBY: Yes.

JOHNSON: Nursery 2, four more children, three with Ashleigh Hudson, one with [Nurse B]?

LETBY: Yes.

JOHNSON: Belinda Simcock with three in nursery 3?

LETBY: Yes.

JOHNSON: And then Angela McShane with three in nursery 4 and [Nurse B] with the fourth baby in nursery 4?

LETBY: Yes.

JOHNSON: So it follows, doesn't it, that the only common nursing staff, the only nursing staff in common between the shift on which [Baby F] received insulin and the shift on which [Baby L] was to receive insulin were you and Belinda Williamson?

LETBY: Yes.

JOHNSON: Now, bearing in mind the question or the issue identified by Professor Hindmarsh as to the timing at which the insulin was put in, overnight, under the care of Tracey Jones, [Baby L] had done very well, hadn't he?

LETBY: Yes.

JOHNSON: And with apologies for having multiple documents open at the same time, if we go to jury bundle number 2, please, so it's the big blue 2 as far as you are concerned, behind divider 15, there at page 17948 we have the dextrose prescriptions, don't we --

LETBY: Yes.

JOHNSON: -- showing Dr Bhowmik's several attempts to resolve the calculation --

LETBY: Yes.

JOHNSON: -- on 8 April and also showing the replacement of the original bag at what you have entered as 12.10 hours?

LETBY: Yes.

JOHNSON: Albeit, I think, in answer to your counsel you said that that actually reads 12.00 hours; is that right?

LETBY: I read it to be that, yes.

JOHNSON: You read it to be 12.00?

LETBY: Yes.

JOHNSON: All right. Moving on in that document, we have the observation chart at 17978; is that right?

LETBY: Yes.

JOHNSON: We have the blood gas record at 17997 with Dr Ukoh's manuscript additions at the back of the form at 17998?

LETBY: It's not blood gases, it's the lab blood results.

JOHNSON: Yes, you're quite right. Then at 18000 we have the blood gas. Then following that we have the printouts, two printouts, from the lab --

LETBY: Yes.

JOHNSON: -- about which we heard quite a lot of evidence.

LETBY: Yes.

JOHNSON: I am going to suggest that you put insulin into [Baby L]'s dextrose bag at about 9.30 or just before 9.30 on 9 April; okay?

LETBY: Okay.

JOHNSON: So if we just keep in mind who is where, if we could keep that on the screen, please, Mr Murphy, and can we go to the neonatal review, please. So if you just put that to one side.

LETBY: Do I need it out still?

JOHNSON: Only if you -- I've invited you to it so if you want to make reference -- I'm just reminding you of what's there if you feel the need to remind yourself of it.

Page 3 of the neonatal review. This shows events that were going on in the unit from the beginning of the shift, doesn't it?

LETBY: Yes.

JOHNSON: And we can see that [Baby L]'s entries are in that sort of salmon pink type shading.

LETBY: Yes.

JOHNSON: And that [Baby M]'s are in the yellow shading, which is used in other cases for the child in question.

LETBY: Yes.

JOHNSON: I just want to concentrate on you and Mary Griffith as you were the two nurses that were in nursery 1 with [Babies L & M]; okay?

LETBY: Yes.

JOHNSON: From 9 o'clock we see an entry for you in relation to the child GT, who is one of the children in nursery 1, don't we?

LETBY: Yes.

JOHNSON: Thereafter, we see that there are four entries between lines 25 and --

MR JUSTICE GOSS: Can we just -- because we haven't got it on the screen, this, so people will be having to look at the paper copy and I don't think everyone's got the paper copy. So this is in the black A3 -- it's...

MR JOHNSON: Number 2.

MR JUSTICE GOSS: About a sixth of the way in. It's for both twins, they're in the same review.

MR JOHNSON: Sorry. Too many balls in the air.

MR JUSTICE GOSS: I think you will need to look at this if you're going to follow this questioning. The alternative is for everyone to turn up the sheet of the layout of the unit and for then the entries to come up on the screen.

MR JOHNSON: Yes, that may be a better alternative.

If we could go to, at least as far as you're concerned, ladies and gentlemen, page 2. I'm going to put page 2 over page 3 so I can see both at the same time.

What I am directing your attention to at the moment is what you were up to between 9 am and 9.30; all right?

LETBY: Okay.

MR JUSTICE GOSS: So this is page 3 in here, in the [Baby L] and [Baby M] neonatal review. It's halfway down, entries 21 and 22 is where we are on page 3.

MR JOHNSON: Yes.

So starting with 21 then, we see an entry made by Mary Griffith on [Baby L]'s neonatal fluid balance chart, don't we?

LETBY: Yes.

JOHNSON: And at the same time you are giving a feed to the child GT?

LETBY: Yes.

JOHNSON: At the same time as you were giving the feed, you appear to be co-signing medication for the patient AW?

LETBY: Yes.

JOHNSON: The child AW is one of the children in nursery 2 who was being looked after by Ashleigh Hudson?

LETBY: Yes.

JOHNSON: Then in the couple of minutes after that, there's some more medication you are co-signing for for AW?

LETBY: Yes.

JOHNSON: And then between lines 29 and 32 inclusive, four entries for the child GT, who was one of your two children in nursery 1?

LETBY: Yes.

JOHNSON: Which you are signing and co-signing with Mary Griffith?

LETBY: Yes.

JOHNSON: Mary Griffith then left the room, didn't she, as we can see from lines 34 down to 39?

LETBY: I can't say that she left the room. Medications are predominantly drawn up in nursery 1. I couldn't say where this happened.

JOHNSON: Well, this is Mary Griffith co-signing with a -- who is Angela McShane?

LETBY: She's a nursery nurse.

JOHNSON: And are nursery nurses allowed to give medications to children?

LETBY: Yes, oral medications.

JOHNSON: Oral medications.

LETBY: Yes.

JOHNSON: These are prescribed medications, aren't they?

LETBY: Yes.

JOHNSON: All signed for between 9.25 and 9.29?

LETBY: Yes.

JOHNSON: And for three children --

LETBY: Yes.

JOHNSON: -- ML, ND and MB?

LETBY: Yes.

JOHNSON: And that was the opportunity that you took to poison [Baby L], wasn't it?

LETBY: No.

JOHNSON: Are you suggesting that staffing levels caused or contributed to somebody putting insulin into the dextrose bag or bags for [Baby L]?

LETBY: No. I don't know how the insulin got there.

JOHNSON: Well, this was -- whoever did it did it deliberately, didn't they?

LETBY: If it happened on the unit, yes.

JOHNSON: Yes. Well, we've already established it has to have happened on the unit, doesn't it, because it happened some time between midnight and about 9.30 --

LETBY: Yes.

JOHNSON: -- in that bag that was connected to [Baby L] throughout that time?

LETBY: Yes, so then when the cannula is replaced, yes.

JOHNSON: And that's why it's a targeted attack, isn't it?

(Pause)

What do you say?

LETBY: Not by me it wasn't.

JOHNSON: If it's not by you, it's by somebody else then, isn't it --

LETBY: Yes.

JOHNSON: -- poisoning a child in the same way that [Baby F] was poisoned --

LETBY: Yes.

JOHNSON: -- with the same substance?

LETBY: Yes.

JOHNSON: And is the reality that unless there's more than one poisoner, it has to have been either you or Belinda Simcock?

LETBY: I can only answer for myself and say that I've never put insulin into any bags.

JOHNSON: It was never suggested to her that she did it though, was it?

LETBY: I can't answer that, I don't know.

JOHNSON: We can put the neonatal review away, please, and if we can go -- I'm sorry to have everybody going to so many different documents, but if we go back to the table that Professor Hindmarsh dealt with in his evidence behind divider 6 in jury bundle 1, so it's the big blue 1 on the back so far as you are concerned. That's it.

You were asked several questions on 16 May about this table; do you remember?

LETBY: Yes.

JOHNSON: I think you started by suggesting that the entry at what you have interpreted for us as being 12.00 hours on 9 April might have been a rate change, but I think you then decided, and I don't mean that in any pejorative way, but you thought that actually that was a change of bag at midday?

LETBY: Yes.

JOHNSON: Just for the jury's benefit, you're looking at the prescription there, aren't you?

LETBY: Yes.

JOHNSON: And that is J -- you just read the red number out for us, please?

LETBY: 17948.

JOHNSON: It's one of the documents behind -- is it divider 10 in jury bundle 2? Divider 12? It's divider 15.

So just so that we're clear about this, it's 17948; is that right?

LETBY: Yes.

JOHNSON: Thank you. So there we see -- it's the entry where I asked you just to confirm what you'd said earlier about not -- you're saying it doesn't say 12.10, it says 12.00 hours; is that right?

LETBY: I would read it as that, yes.

JOHNSON: Ten minutes either way in this circumstance probably doesn't make any difference, but this, in any event, shows a different bag and I think we heard from several witnesses that the practice is to change a dextrose bag every 24 hours anyway.

LETBY: Yes.

JOHNSON: But the giving set doesn't change; is that right?

MR MYERS: That's --

MR JOHNSON: Well, I'm asking the question. Am I not allowed to ask that question?

MR MYERS: We heard in the evidence that the dextrose is changed every 24 hours.

MR JUSTICE GOSS: 24 or 48. It's not universally --

LETBY: Dextrose is.

MR JUSTICE GOSS: Sorry? I'll stop. You say. You're the witness.

LETBY: Dextrose is changed 24-hourly without fail. It's TPN that's 48-hourly.

MR JOHNSON: That's what I thought.

MR MYERS: I only rose because it was said we had heard from the evidence it was changed every 24 hours and the giving sets were changed. I'm just putting (overspeaking) giving sets were not changed and I rise because in fact, whatever the question is, that is not what the evidence has been on the changing of sets.

MR JUSTICE GOSS: Well --

MR MYERS: It's important.

MR JUSTICE GOSS: It is, Mr Myers. I think the answer is that the witness can speak, if she has a recollection, of what happened on this occasion. That is the important thing.

MR MYERS: Yes. I make no issue with that. It's when evidence is read out --

MR JUSTICE GOSS: It wasn't clear -- in fact, it wasn't unequivocal that the giving set was not changed is my recollection.

MR JOHNSON: Not in dextrose.

MR JUSTICE GOSS: But anyway, be that as it may.

MR JOHNSON: Well, you tell us: the changing of giving sets with dextrose, what's the policy?

LETBY: That it would be changed.

JOHNSON: Every time?

LETBY: Yes. I don't -- sorry, I'm confused with what you're asking now.

JOHNSON: A dextrose bag, as I understand you to have told us, is changed every 24 hours?

LETBY: Yes.

JOHNSON: The bag is hooked up to a giving set?

LETBY: Yes.

JOHNSON: Connected to a giving set?

LETBY: Yes.

JOHNSON: Does the giving set have to be changed every time a bag of dextrose is changed?

LETBY: Not every time, no.

JOHNSON: No. Well, that actually was the proposition I was putting to you, but there we are, we got there.

LETBY: Sorry, I misunderstood.

JOHNSON: It's all right, it's not your fault.

Despite the fact that the bag was changed, so looking at the table that Professor Hindmarsh took us through, despite the fact that the bag was changed at midday, 12.00 hours, the insulin kept being administered to [Baby L], didn't it?

LETBY: Um... Yes.

JOHNSON: Yes. And we know that because, if we look at page 2, we see the time at which the blood sample was received in the lab being either 18.26 or 18.29.

LETBY: Yes.

JOHNSON: And that blood sample contained exogenous insulin, didn't it?

LETBY: I can't recall.

JOHNSON: Well, that was the evidence from the lab.

LETBY: Okay.

JOHNSON: Do you remember? It's the -- just going to the documents that are in front of you in the other file, they're the printout documents that are near the end.

Just before that, actually. Keep going towards the front from where you are. There, that one. That's where I get the time from; okay? So I'm going to suggest, for your comment or your answer, that not only was [Baby L] targeted on the first occasion whilst the bag was hanging, but he also got a second bag that had insulin in it. What do you say about that?

LETBY: I'd have to be guided by the evidence, the expert evidence.

MR JUSTICE GOSS: Sorry, I couldn't hear that.

MR JOHNSON: "I'd have to be guided by the evidence."

MR JUSTICE GOSS: You'd have to be guided by the evidence and then you said something else and I couldn't --

LETBY: The expert evidence.

MR JUSTICE GOSS: You have to be guided by the expert evidence? That's what I couldn't hear.

MR JOHNSON: We have a third bag hung, don't we, at about 16.30 or thereabouts?

LETBY: Yes.

JOHNSON: The second row on the second page of that table. This was -- sorry, it's in the one --

LETBY: Sorry.

JOHNSON: Divider 6. That's it. You've got it there.

LETBY: Yes, sorry.

JOHNSON: So a third bag hung at 16.30 and the hypoglycaemia continued?

LETBY: Yes.

JOHNSON: Fourth bag hung the following day when you were not working on 10 April at -- it's a 15% bag at either 2.30 or 3.00 hours?

LETBY: Yes.

JOHNSON: And the hypoglycaemia gradually resolved, didn't it?

LETBY: Yes.

JOHNSON: We end up, as we go from the 10th into the 11th, 23.00 hours on the 11th, which is right at the end of page 3, with a resolved episode of hypoglycaemia?

LETBY: Yes.

JOHNSON: And the reason for the hypoglycaemia was that somebody had poisoned at least two glucose bags with insulin, isn't it?

LETBY: Yes.

JOHNSON: And that was you, wasn't it?

LETBY: No.

MR JOHNSON: I'm going to move on to [Baby M] now, my Lord. It's 2 minutes to 1.

MR JUSTICE GOSS: Yes. We'll break off there, members of the jury. Could you be ready, please, to be back in court for 2.10 or a minute or two after that? That's nearly an hour and a quarter. Please remember your responsibilities: don't discuss the case with anyone unless you're all 12 together and no research.

(12.59 pm)

(The short adjournment)

(2.08 pm)

(In the absence of the jury)

Housekeeping

MR JUSTICE GOSS: Mr Johnson, Mr Myers, regrettably, for reasons that you are aware of, it will not be possible to continue with the trial this afternoon, nor will it be possible to proceed tomorrow either, but I'm hoping that tomorrow afternoon there will be information to say when we will resume the case. Just so that there's no mystery about it, a juror is not well and requires, effectively, emergency surgery, but it's not known how long it will be before that juror will be able to continue.

So it's entirely beyond anyone's control, this, and there's no option but to say not sitting today. The jury have already been sent home, the other jurors, because they were aware of the difficulty anyway.

We will have an update tomorrow afternoon. That being Wednesday, that leaves Thursday and Friday of this week, in the best case scenario. Monday, Tuesday, we are not sitting in any event with the jury, or Friday, of next week.

MR JOHNSON: Yes.

MR JUSTICE GOSS: So obviously, I'm very sorry about this.

MR MYERS: My Lord, we are grateful for the information.

Naturally, anybody hearing about the position of the juror will have sympathy with the juror and it must be a matter of acute concern to be rushed for surgery right now. We recognise that.

Also, I have to express, we have concerns, naturally, for the position of Ms Letby, who has been giving evidence now since 2 May.

MR JUSTICE GOSS: I know. It's been very disjointed.

MR MYERS: It is. The pressures are significant in all the circumstances and upon her. And we recognise there's the balance between accommodating the juror, but at the same time proceeding, and obviously we can't today and your Lordship requires further information.

MR JUSTICE GOSS: When we have the information tomorrow afternoon, that will be a time when I think we can revisit the future.

MR MYERS: Yes.

MR JUSTICE GOSS: I don't think it's appropriate to do it now.

MR MYERS: We don't seek to do that. What we were going to suggest, my Lord, though it's a matter for the court and what can be achieved, it may be helpful if some type of formal assessment were available to your Lordship, because we don't know the nature of the treatment and there's no doubt the jury, so far as all those of us in court can follow, are committed to this court, but of course we need to know that any juror who is here is well able to do their job properly.

MR JUSTICE GOSS: Oh, absolutely. I have all these matters well in mind, Mr Myers, not least the disruption there has already been and will continue to be to the defendant giving evidence in this case.

The issue then is whether there should be a formal hearing tomorrow afternoon.

MR MYERS: It may be of some assistance for us to be here, my Lord, because we don't know what information there will be yet or the quality of that information.

MR JUSTICE GOSS: No.

MR MYERS: And if we are convened we can assess it more thoroughly, I respectfully suggest, than if we try to do so at a distance from one another. The defendant will understand what is happening, she can be present --

MR JUSTICE GOSS: Yes.

MR MYERS: -- as your Lordship would doubtless wish.

Therefore we can look at the matter fully with everybody present and take whatever action is or is -- not take any action if none is required.

MR JUSTICE GOSS: Absolutely. I think there should be a hearing tomorrow afternoon.

MR MYERS: We'd be grateful, my Lord.

MR JUSTICE GOSS: Because I hope and expect to be far better informed than I am now and I think that we will have, as you have referred to, some detailed information that will enable me to consider what happens after tomorrow.

MR MYERS: That's what we wanted to identify. I'm grateful, my Lord, thank you.

MR JUSTICE GOSS: Well, thank you.

Mr Johnson, is there anything you wish to add to that?

MR JOHNSON: No, thank you.

MR JUSTICE GOSS: As matters presently stand, may I say that the messages I have received during the course of the trial in relation to misfortunes that have befallen jurors is that each and every one of them has been wholly committed to wanting to continue to serve on this jury. And for all sorts of reasons, insofar as the jury can be preserved as a jury of 12, they should be. So that is what is happening.

As the defendant has heard me say, I'm very much aware of and alive to the added stress that this is causing to her.

MR MYERS: Very well, my Lord. Thank you.

MR JUSTICE GOSS: But there we are. All right?

MR MYERS: Thank you.

MR JUSTICE GOSS: And of course, to everyone else who's concerned with this case, the fact that -- there are a lot of people with a lot of personal interests --

MR MYERS: Yes.

MR JUSTICE GOSS: -- and they will be, I hope, understanding of the situation and what has to be done.

MR MYERS: Very well, my Lord.

MR JUSTICE GOSS: I think if the case is listed for 2.15 tomorrow afternoon, so as far as you are concerned, madam, to be here for a hearing to commence at 2.15 tomorrow afternoon, please.

(Pause)

2.15, please. Not the normal time in other words, but in good time for 2.15. I'll leave that to you, thank you very much.

(Pause)

MR JUSTICE GOSS: Maybe I misheard or misunderstood that.

The defendant was...

MS CLANCY: I think the officer was asking if she could see the liaison and --

MR JUSTICE GOSS: Oh, Miss Santolini, yes.

MS CLANCY: Because it's an earlier finish, but we've indicated to the officer that she's available.

MR JUSTICE GOSS: Good. Ms Santolini is back? Because she wasn't here yesterday, was she?

MS CLANCY: Yes, she was actually here today, I think that was my mistake.

MR JUSTICE GOSS: Thank you for arranging that anyway.

(2.16 pm)

(The court adjourned until 2.15 pm on Tuesday, 6 June 2023)